

Drug Management Policy

Tremfya

Policy Number: 9.150

Version Number: 6

Version Effective Date: 07/29/2025

Plan		
☐ MassHealth ACO	☒ MA Clarity plans	☐ MA Senior Care Options
☐ MassHealth MCO	☐ NH Clarity plans	☐ NH Medicare Advantage
☐ NH Medicaid		
Benefit		
☒ Pharmacy	☐ Medical	

Note: Disclaimer and audit information is located at the end of this document.

Prior Authorization Policy

Products Affected:

- **Tremfya** (guselkumab)

The Plan may authorize coverage of the above products for members meeting the following criteria:

Covered use: All medically accepted indications unless otherwise excluded

Moderate to severe plaque psoriasis (Ps)

1. The member is 18 years of age or older; **AND**
2. The requested medication is prescribed by or in consultation with a dermatologist; **AND**
3. ONE of the following:
 - a. Involvement of at least 3% of total body surface area prior to the use of the requested medications; **OR**

- b. Hands, feet, scalp, face, or genital area affected; **AND**
- 4. ONE of the following:
 - a. An inadequate response or adverse reaction to one biologic DMARD that is FDA-approved for plaque psoriasis; **OR**
 - b. An inadequate response or adverse reaction to at least a 3 consecutive month trial of any one of the following combinations (please note: these combinations DO NOT have to be used concurrently)
 - i. one topical agent plus one systemic agent; **OR**
 - ii. one topical agent plus one phototherapy; **OR**
 - iii. one systemic agent plus one phototherapy; **OR**
 - iv. two systemic agents; **OR**
 - c. A contraindication to methotrexate, as determined by the prescriber; **AND**
- 5. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A).

Psoriatic Arthritis (PsA)

- 1. The member is 18 years of age or older; **AND**
- 2. The requested medication is prescribed by or in consultation with a rheumatologist; **AND**
- 3. ONE of the following:
 - d. An inadequate response or adverse reaction to at least a 3 consecutive month trial of one traditional disease modifying antirheumatic drug (DMARD) or contraindication to all traditional DMARDs; **OR**
 - e. An inadequate response or adverse reaction to one biologic DMARD that is FDA-approved for psoriatic arthritis; **AND**
- 4. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A).

Moderately to Severely Active Ulcerative Colitis (UC)

- 1. The member is 18 years of age or older; **AND**
- 2. One of the following:
 - a. Member has tried one systemic therapy (such as 6- mercaptopurine, azathioprine, cyclosporine, tacrolimus, a corticosteroid such as prednisone or methylprednisolone, or a previous trial of a biologic); **OR**

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- b. Member has pouchitis and has tried an antibiotic (such as metronidazole or ciprofloxacin), probiotic, corticosteroid enema, or mesalamine enema; **AND**
- 3. The requested medication is prescribed by or in consultation with a gastroenterologist; **AND**
- 4. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A)

Moderate to severely active Crohn's disease (CD)

- 1. The member is 18 years of age or older; **AND**
- 2. The medication is prescribed by or in consultation with a gastroenterologist; **AND**
- 3. One of the following:
 - a. a. Member has tried or is currently taking corticosteroids, or corticosteroids are contraindicated in this member; **OR**
 - b. b. Member has tried one other conventional systemic therapy for Crohn's disease (such as azathioprine, 6- mercaptopurine, methotrexate, or a previous trial of a biologic; Note: a trial of mesalamine does not count); **OR**
 - c. Member has enterocutaneous (perianal or abdominal) or rectovaginal fistulas; **OR**
 - d. Member had ileocolonic resection (to reduce the chance of Crohn's disease recurrence); **AND**
- 4. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A)

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Reauthorization criteria

- 1. Initial criteria has been met; **AND**
- 2. One of the following:
 - a. The member has had a positive response to therapy; **OR**
 - b. Documentation of clinical justification to continue therapy with the requested medication

Coverage Duration

Initial: 6 months

Reauthorization: 12 months

Appendix A- FDA Approved Dosing

Diagnosis	FDA Approved Dosing
Plaque Psoriasis	100 mg subcutaneous injection at Week 0, Week 4 and every 8 weeks thereafter.
Psoriatic Arthritis	100 mg subcutaneous injection at Week 0, Week 4 and every 8 weeks thereafter.
Ulcerative Colitis	<u>Induction:</u> 200mg intravenous infusion at Week 0, Week 4, and Week 8 <u>Maintenance:</u> 100mg subcutaneous injection at Week 16 and every 8 Weeks thereafter OR 200mg subcutaneous injection at Week 12 and every 4 Weeks thereafter
Moderate to severely active Crohn's disease (CD)	<u>Induction:</u> 400 mg (as 2 consecutive 200 mg injections) on weeks 0, 4, and 8 <u>Maintenance:</u> 100 mg every 8 weeks beginning at week 16 or 200 mg every 4 weeks beginning at week 12.

Appendix B - Non-Biologic DMARD Treatment Options

Diagnosis	Non-Biologic DMARD Treatment Options
Psoriatic Arthritis	Methotrexate Leflunomide Sulfasalazine Azathioprine
Plaque Psoriasis	Methotrexate Azathioprine

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Cyclosporine

Note: other trials may be considered on a case-by-case basis

Code	Medication
J1628	Injection, guselkumab 1 mg

Clinical Background Information and References

1. Elmetts, C. Korman, N. Farley Prater, E. et. al. Joint AAD–NPF Guidelines of care for the management and treatment of psoriasis with topical therapy and alternative medicine modalities for psoriasis severity measures. Journal of the American Association of Dermatology. 84(2): p432–470. Feb. 2021. Accessed June 2024.
2. Elmetts, C. Lim, H, Wu, J. et. al. Joint American Academy of Dermatology–National Psoriasis Foundation guidelines of care for the management and treatment of psoriasis with phototherapy. Journal of the American Association of Dermatology. 81(3): p775–804. Sept. 2019. Accessed June 2024.
3. Galdman, D. Ritchlin, C. Treatment of psoriatic arthritis. UpToDate. May 2023. Accessed June 2024.
4. Tremfya (guselkumab) [prescribing information]. Horsham, PA: Janssen Biotech Inc. July 2020. Accessed June 2024.
5. Menter A, Strober BE, Kaplan DH, et al. Joint AAD–NPF guidelines of care for the management and treatment of psoriasis with biologics. J Am Acad Dermatol 2019; 80:1029–72. Accessed June 2024.
6. Singh JA, Guyatt G, Ogdie A, et al. 2018 American College of Rheumatology/National Psoriasis Foundation Guideline for the Treatment of Psoriatic Arthritis. Arthritis & Rheumatology. 2019; Jan;71(1):5–32. Accessed June 2024.

Policy History

Original Approval Date	Original Effective Date	Policy Owner	Approved by
5/24/2021	7/1/2021	Pharmacy Services	Pharmacy & Therapeutics (P&T) Committee

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Policy Revisions History				
Version	Review Date	Summary of Revisions	Effective Date	Approved by
1	5/24/2021	New policy created.	7/1/2021	Pharmacy & Therapeutics (P&T) Committee
2	8/12/2021	P&T Annual Review: Addition of 3-month trial time requirement; Addition of diagnostic criteria and change to non-biologic DMARD contraindication from all to MTX only for Ps to align with ESI ICCV guidelines.	1/1/2022	Pharmacy & Therapeutics (P&T) Committee
3	8/11/2022	P&T Annual Review: Clarified diagnosis of Ps to include "moderate to severe"; Updated approval duration for initial approvals from 12 to 6 months; Removed reauthorization criteria; Added dermatologist as an acceptable prescriber for PsA; Minor formatting updates; Added J code; Updated to new template.	12/1/2022	Pharmacy & Therapeutics (P&T) Committee
4	8/10/2023	P&T annual review: Addition of dosing criteria and appendix Updated reauthorization criteria to standard required for all biologics in same class.	12/1/2023	Pharmacy & Therapeutics (P&T) Committee
4	8/8/2024	P&T annual review: No recommended changes.	12/1/2024	Pharmacy & Therapeutics (P&T) Committee
5	2/18/2025	Policy updated for UC indication	3/19/2025	Pharmacy & Therapeutics (P&T) Committee

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Policy Revisions History				
6	6/25/2025	Added criteria for new FDA indication of Moderate to severely active Crohn's disease as part of IACCV program	7/29/2025	IACCV Program

Next Review Date

8/2025

Reference to Applicable Laws and Regulations, If Any

Disclaimer Information

Medical Policies are the Plan's guidelines for determining the medical necessity of certain services or supplies for purposes of determining coverage. These Policies may also describe when a service or supply is considered experimental or investigational, or cosmetic. In making coverage decisions, the Plan uses these guidelines and other Plan Policies, as well as the Member's benefit document, and when appropriate, coordinates with the Member's health care Providers to consider the individual Member's health care needs.

Plan Policies are developed in accordance with applicable state and federal laws and regulations, and accrediting organization standards (including NCQA). Medical Policies are also developed, as appropriate, with consideration of the medical necessity definitions in various Plan products, review of current literature, consultation with practicing Providers in the Plan's service area who are medical experts in the particular field, and adherence to FDA and other government agency policies. Applicable state or federal mandates, as well as the Member's benefit document, take precedence over these guidelines. Policies are reviewed and updated on an annual basis, or more frequently as needed. Treating providers are solely responsible for the medical advice and treatment of Members.

The use of this Policy is neither a guarantee of payment nor a final prediction of how a specific claim(s) will be adjudicated. Reimbursement is based on many factors, including member eligibility and benefits on the date of service; medical necessity; utilization management guidelines (when applicable); coordination of benefits; adherence with applicable Plan policies and procedures; clinical coding criteria; claim editing logic; and the applicable Plan – Provider agreement.